TRT-GUY

TRT VS. SPORTS TRT/HRT (OR TRT+) VS. RUNNING STEROID CYCLES

This is a simple guide that breaks down the differences between dosing approaches for testosterone and AAS, how I think about moving between them, and how to approach long-term use while keeping health in the picture.

Outside of clinical TRT, the terms presented here aren't official medical terms. They're simply a framework to help you understand what each approach is trying to accomplish.

Much of what I'll explain comes from what I've learned from Broderick Chavez of Team Evil GSP. I highly recommend checking out his work if you're interested in PED and sports-performance education.

DISCLAIMER

The use of testosterone and other anabolic steroids is detrimental to your health. This guide is for education and entertainment, not a personal prescription or a recommendation to use them.

I don't condone or recommend the use of any illegal substances or drugs, and doing so without proper medical oversight will probably result in negative health outcomes that you will regret later on.

HOW TO USE THIS GUIDE

The guide is organized around a simple progression:

1. **Understand the level of androgen exposure.**
2. **Define what you are trying to accomplish.**
3. **Choose an approach that matches the goal and timeline.**
4. **Monitor your health.**
5. **Adjust based on your response.**
6. **Use training, nutrition, sleep, cardio, and recovery to get more from the dose you are already using.**

PART I

UNDERSTANDING ANDROGEN EXPOSURE

CLINICAL TESTOSTERONE REPLACEMENT THERAPY — TRT

TRT is a medical treatment used to restore normal testosterone levels in men with hypogonadism.

Clinical TRT is adjusted using blood levels, symptoms, and adverse effects; the Endocrine Society lists 75–100 mg per week of enanthate or cypionate as a starting regimen.

> **We can round that out to approximately 75–125 mg of testosterone per week as a testosterone replacement dose. That being said, the final dose will have to be adjusted based on bloodwork and symptom resolution.**

A man with low testosterone who starts TRT may experience:

- Mood improvement.
- Greater resilience to stress.
- Improved libido and sexual function.
- Increased muscle mass.
- Reduced body fat.
- Better energy.
- Better focus.
- Increased work capacity.

The goal is to replace what's missing and improve the symptoms of low testosterone.

This will feel like a huge enhancement initially if you're truly hypogonadal.

SPORTS TRT/HRT (OR TRT+)

These aren't medical terms. They're terms that Broderick Chavez came up with that I now use to distinguish a clinical TRT protocol from what I call "a juicier long-term protocol."

The goal of sports TRT is not only to replace endogenous testosterone production but also to maximize sports performance.

> **In the framework I learned, sports TRT/HRT falls within 2–3 mg/kg per week.**

You're aiming for more androgen exposure than replacement alone would provide.

Think even greater potential for:

- Even better mood.
- Improved sexual function.
- Muscle gain.
- Increased strength.
- Body composition improvements.
- Improved cognitive function.
- Increased work capacity.
- More confidence and drive.

> **NOTE: Moving from a medical TRT dose to a sports TRT/HRT dose substantially increases your androgen exposure. At this point, the goal is no longer simply replacement; you're deliberately moving into a supraphysiological range.**

WHAT'S THE DIFFERENCE BETWEEN SPORTS TRT AND SPORTS HRT?

Sports Testosterone Replacement Therapy (TRT) → Testosterone alone

Some men will not be able to run 2–3 mg/kg per week of testosterone alone without experiencing negative side effects. In that case, moving toward a sports HRT protocol would make more sense.

Sports Hormone Replacement Therapy (HRT) → Testosterone + AAS

Examples for a 100 kg man:

- Sports TRT → 200–300 mg of testosterone per week.
- Sports HRT → 150 mg of testosterone enanthate + 150 mg of Masteron per week.

HOW DO YOU SELECT THE OTHER COMPOUND AND THE RATIO?

There is no straightforward answer, and this needs to be evaluated on a case-by-case basis.

There's no single ratio that works for everyone.

The reasoning depends on your goals, individual response, and the effects that each compound has on you.

AAS commonly used in sports HRT include:

- Masteron.
- Primobolan.
- Nandrolone.
- Boldenone.

EXAMPLES OF SPORTS HRT

For a 100 kg male:

- 100 mg of testosterone + 200 mg of Primobolan or Masteron per week.
- 150 mg of testosterone + 150 mg of Primobolan or Masteron per week.
- 200 mg of testosterone + 100 mg of Primobolan or Masteron per week.

MY PERSONAL PREFERENCE

Personally, I've only used Masteron or Primobolan as part of my sports HRT protocols. In my experience, both worked better for me than testosterone alone and produced similar recovery and anabolic effects with less water retention and bloating – given that I'm genetically prone to high blood pressure, I favored using them instead of testosterone alone.

That being said, I felt my best, and got my best results with a combination of testosterone and Masteron as my Sports HRT protocol.

My experience cannot necessarily be extrapolated to you.

People respond differently to different compounds, so any decision has to take into account your goals, circumstances, health markers, and individual response.

PART II

HIGHER-DOSE CYCLES

RUNNING HIGHER-DOSE CYCLES

Here, we're talking about a planned period of steroid use with even higher androgen exposure, usually with the idea of accomplishing a specific goal, and I'll give you a simple framework so you have something to work with.

In the framework I'm using here, I consider it a "cycle" once total AAS exposure reaches roughly 5 mg/kg per week.

> **Example for a 100 kg man → 500 mg of total AAS per week.**

Here are a few questions to ask yourself before starting a cycle:

1. WHAT ARE YOU TRYING TO ACCOMPLISH?

Start with a clear goal.

Examples:

Are you trying to build muscle, lose fat and get visible abs, improve a specific skill, get better at a sport, or improve cardiovascular capacity?

These are some examples of potential goals, so think hard about what you want to achieve.

Each goal calls for different approaches to training, nutrition, recovery, and drug selection. There are OTHER PEDs aside from AAS that could help you achieve certain goals.

For example, modafinil can help with improved skill acquisition and increased energy expenditure, clenbuterol and HGH can help with fat oxidation, and melanotan 2 can help you tan, increase cardiovascular capacity, and improve erection quality.

2. WHAT'S YOUR TIMELINE?

Once the goal is clear, establish a realistic date for when you'd like to achieve it.

The timeline should reflect what you can reasonably accomplish.

When it comes to AAS use, I like to work in 16-week cycles. This is usually enough time to make significant progress toward a specific goal, and it's usually the threshold where things start to break down and a break is warranted to recover and reevaluate.

Once you finish your 16-week cycle, you will enter a bridge phase, or cruise phase. This is a stage where you lower your androgen load to either a Sports TRT/HRT or a clinical TRT dose, and you could use this time to work toward other goals that will allow you to push again later on.

The bridges (or cruise phases) should be at least ½ the duration of your cycle, but if you want to be a bit more conservative, you could do this on a 1:1 basis, where you run a 16-week cycle and then run a 12-16-week bridge.

Some examples of goals you could pursue during cycles: acquiring muscle mass, or improving a certain muscle group (more legs, upper body, shoulders, etc.).

Some examples of goals you could pursue during bridges: fat loss and improving your cardiovascular capacity. These periods of time should also be used to restore health markers that are in negative ranges.

All of this needs to be evaluated on an individual basis. This is a simple framework to help you start thinking about long-term AAS use.

3. WHAT PEDS WILL HELP YOU ACHIEVE YOUR GOALS?

This is where working with a knowledgeable coach comes in handy.

Different AAS can produce different short-term changes in body weight, water retention, appearance, and how you feel.

And, as explained above, there are other non-AAS drugs you can use to achieve your goals.

For a simple example, consider two men who want to build muscle and improve their body composition:

- An overweight man who already has high blood pressure.
- A 190 cm, 60 kg man who wants to gain body weight.

Their starting points are very different. For the first man, additional fluid retention could worsen an existing problem, so selecting drugs that drastically increase water retention wouldn't be the smartest choice.

For the second, a higher number on the scale might be one of his goals. In this case, extra water retention would be a positive, so you could use drugs that create more water retention.

PART III

SAMPLE AAS PROTOCOLS

The protocols below are included because I want this guide to be practical. They show how I think about structuring androgen exposure across different goals and time periods.

EXAMPLE #1: ORAL-ONLY APPROACH – DIPPING YOUR TOES

If you specifically want to explore a short period of supraphysiological androgen exposure without committing to ongoing testosterone injections, this is one approach discussed in this framework.

These are some reasons why you'd want to start with an oral-only cycle:

1. You want to experience higher androgen exposure without committing to ongoing injections.
2. You want to keep the number of drugs involved to a minimum.
3. You want to keep the duration and number of compounds limited, while recognizing that any AAS exposure can suppress natural testosterone production.
4. Injectable testosterone suppresses endogenous testosterone production, and commonly used esters such as enanthate and cypionate remain active longer than many short-acting oral compounds.
5. You want a compound that becomes active relatively quickly and clears relatively quickly after you stop taking it.
6. For those priorities, a shorter-acting compound may seem more appealing, although shorter duration does not mean low risk.
7. Oxandrolone is not testosterone, but that does not mean it is non-suppressive or free of significant adverse effects. The rationale for this approach is to use a single, shorter-acting AAS rather than adding injectable testosterone.

Here’s the cycle:

TOTAL DURATION	**9 weeks**
MAXIMUM ANDROGEN LOAD	210 mg per week
RECOMMENDATIONS	Get bloodwork before running the cycle to establish baseline health markers, and then get it again at the end to see what happened.
COMPOUND USED	Oxandrolone (Anavar)

WEEKS	DOSE
Weeks 1–3	Anavar: 70 mg per week (10 mg per day taken at any time)
Weeks 4–6	Anavar: 140 mg per week (20 mg per day taken at any time)
Weeks 7–9	Anavar: 210 mg per week (30 mg per day taken at any time)
Week 10 Onward	Nothing.

What will happen after you finish?

You will experience some low testosterone symptoms for a while and will probably not feel like your normal self for a few weeks, but you will need to give your body some time to fully restart your natural testosterone production.

During this time where you’re feeling off, you will keep on training and eating sufficient calories in order to keep some of the tissue gained.

Winstrol is another oral AAS sometimes used in place of Anavar.

If you’re already using injectables, I’d skip this section.

EXAMPLE #2: INJECTABLES ONLY

LENGTH	**16 weeks**
MAXIMUM DOSE	500 mg of total AAS per week
COMPOUND SELECTION	Testosterone enanthate/cypionate + Primobolan or Masteron
RECOMMENDATIONS	Get bloodwork before running the cycle to establish baseline health markers.

WEEKS	WEEKLY DOSE
Weeks 1–3	250 mg of testosterone enanthate/cypionate + 100 mg of Primobolan or Masteron per week.
Weeks 4–7	250 mg of testosterone enanthate/cypionate + 200 mg of Primobolan or Masteron per week.
Weeks 8–16	250 mg of testosterone enanthate/cypionate + 250 mg of Primobolan or Masteron per week.
Week 17 Onward	Back to Sports TRT/HRT or Medical TRT/HRT.

Get bloodwork at weeks 10–12 and at the end of the cycle.

EXAMPLE #3: INJECTABLE + ORALS

LENGTH	**16 weeks**
MAXIMUM DOSE	1,030 mg of total AAS per week
COMPOUND SELECTION	Testosterone enanthate/cypionate + Primobolan or Masteron + Oxandrolone or Winstrol
RECOMMENDATIONS	Get bloodwork before running the cycle to establish baseline health markers.

WEEKS	WEEKLY DOSE
Weeks 1–2	250 mg of testosterone enanthate/cypionate + 200 mg of Primobolan or Masteron per week.
Weeks 3–4	250 mg of testosterone enanthate/cypionate + 300 mg of Primobolan or Masteron per week.
Weeks 5–7	250 mg of testosterone enanthate/cypionate + 400 mg of Primobolan or Masteron per week.
Weeks 8–10	250 mg of testosterone enanthate/cypionate + 500 mg of Primobolan or Masteron per week.
Weeks 11–16	250 mg of testosterone enanthate/cypionate + 500 mg of Primobolan or Masteron + 280 mg Anavar/Winstrol (40 mg per day) per week.
Week 17 Onward	Back to Sports TRT/HRT or Medical TRT/HRT.

Get bloodwork at weeks 10–12 and at the end of the cycle.

INJECTION FREQUENCY

Split your total AAS load over 3–5 weekly injections and rotate sites so you avoid the buildup of scar tissue.

For example, if you're injecting 250 mg of testosterone + 200 mg of another AAS in total for the week, you can divide this into 3 injections.

Assuming your testosterone is 250 mg/mL and your other drug is 200 mg/mL, then this is how you'd dose the 3 injections per week:

DAY	INJECTION
Day 1	0.4 mL of testosterone + 0.3 mL of Primobolan.
Day 2	0.3 mL of testosterone + 0.4 mL of Primobolan.
Day 3	0.3 mL of testosterone + 0.3 mL of Primobolan.

PART IV

HEALTH MONITORING

BLOODWORK TO GET BEFORE AND AFTER A CYCLE FOR COMPARISON

Get these drawn before running anything, repeat every 8–12 weeks, and compare. When you get labs done, it is a good idea to read them or get interpreted by a knowledgeable coach or practitioner that is familiar with steroid use and reading bloodwork.

BEFORE → DURING → AFTER

The simplest way to think about monitoring is to establish a baseline, repeat the relevant markers during use, and compare them over time.

COMPREHENSIVE LAB WORK LIST

01 COMPREHENSIVE METABOLIC PANEL (CMP)

- Electrolytes
- BUN
- Creatinine
- Glucose
- Albumin
- Total Protein
- Bilirubin
- ALT
- AST
- ALP
- Calcium

02 COMPLETE BLOOD COUNT (CBC)

- Hemoglobin
- RBC
- Hematocrit
- WBC
- WBC Differential
- Platelet Count

03 LIPID & CARDIOVASCULAR

- Total Cholesterol
- LDL
- HDL
- Triglycerides
- ApoB
- ApoA1

04 IRON PANEL

- Iron
- TIBC
- UIBC
- Transferrin Saturation
- Ferritin

05 KIDNEY HEALTH

- Cystatin C
- Cystatin C–based eGFR
- Urinalysis
- Urine Albumin/Creatinine Ratio

06 THYROID

- TSH
- Free T4 (FT4)
- Free T3 (FT3)
- Reverse T3

07 HORMONES

- Total Testosterone
- Free Testosterone
- Ultrasensitive Estradiol
- SHBG
- Prolactin
- Pregnenolone
- DHEA-S

08 LIVER

- GGT

09 METABOLIC & INFLAMMATION

- Hemoglobin A1C
- Fasting Insulin
- High-Sensitivity C-Reactive Protein (hs-CRP)
- Homocysteine

10 NUTRIENTS / MINERALS

- 25-OH Vitamin D
- RBC Magnesium

11 PROSTATE

- PSA Total
- Free PSA
- % Free PSA (when appropriate)

PART V

MANAGING COMMON PROBLEMS

AAS EMERGENCY KIT & STRATEGIES

WHAT TO DO IF YOU GET AN INFECTED INJECTION SITE:

If you suspect an injection-site infection, seek immediate medical attention so the site can be assessed and the appropriate treatment can be determined.

Untreated infections from intramuscular injections can turn into sepsis, which is a deadly condition. So if you ever suspect that you have an infection, get medical help immediately.

Personal note: I once had an infection after an intramuscular injection. Within just one week of the symptoms beginning, it had progressed to the point where I was almost septic and required six surgical procedures to remove the necrotic tissue.

I was left with a wound roughly the size of a footlong Subway sandwich, which took almost 4.5 months to fully heal.

> **Moral of the story: don't take infections lightly. A seemingly simple infection can become life-threatening very quickly.**

BLOOD PRESSURE

Cardio is one of the most useful lifestyle strategies for improving cardiovascular health and supporting blood-pressure control.

If your blood pressure is repeatedly elevated, confirm the readings with multiple measures over several days, look at the underlying causes, and work with a physician on treatment when medication is appropriate.

If you decide to self-medicate, either Telmisartan at 20-40 mg/day or Lisinopril Oral 10-40mg once daily would be the first line of action.

That being said, you should seek medical attention and work with your primary care physician to lower your blood pressure if you do need it.

OUT-OF-RANGE HEMOGLOBIN (>18) AND HEMATOCRIT (>54)

If hemoglobin and/or hematocrit are elevated, the goal should be to identify and address the underlying factors before simply adding medications or supplements.

LIFESTYLE RECOMMENDATIONS

- If you have high body fat levels, start dieting and work toward a leaner body composition.
- Stay well hydrated, as dehydration can artificially elevate hematocrit.
- Aim for 100–150 minutes of Zone 2 cardio per week.
- If you smoke, stop.
- Get a sleep study if appropriate and address sleep apnea if present.
- Eat plenty of fruits and vegetables, aiming for approximately 2–3 lb per day.
- Monitor your blood pressure regularly.

SUPPLEMENTS / PHARMACEUTICALS TO CONSIDER

- Nattokinase: 4,000–6,000 FU/day
- Curcumin: 500–1,000 mg/day
- High-quality omega-3: 2–4 g/day
- IP6 if ferritin is elevated

ADDITIONAL INTERVENTIONS IF NEEDED

- Consider an ARB such as telmisartan if clinically indicated.
- Consider short-term AI use to manage elevated estradiol if appropriate and until the underlying issue is addressed.

ADDITIONAL CONSIDERATIONS

- If there is a personal or family history of DVT/thrombosis, consider evaluating for underlying clotting disorders, including fibrinogen and Factor V Leiden testing.

> **Most importantly: Don't simply add medications or supplements because a marker is out of range. Work with your primary care physician, TRT provider, or coach to identify the reason for the elevation and determine which interventions are appropriate.**

ACNE ON TRT/PEDS

If you're experiencing acne while using TRT/PEDs, there are several factors worth addressing before jumping straight to medication.

LIFESTYLE RECOMMENDATIONS

- Keep your skin clean, particularly after training or sweating.
- Avoid staying in sweaty clothes for extended periods.
- Change your bed sheets regularly, ideally at least once per week.
- Avoid harsh scrubbing, which can irritate the skin and increase inflammation.
- Prioritize 7–9 hours of high-quality sleep.
- Manage stress through practices such as meditation, journaling, or other stress-reduction strategies.
- Keep your diet centered around minimally processed foods and consider reducing foods that may aggravate acne, such as dairy and highly processed foods.
- Eat plenty of fruits and vegetables to support overall nutritional and antioxidant intake.
- Consider increasing testosterone injection frequency, such as moving to daily injections, to reduce hormonal fluctuations.

NUTRITIONAL / SUPPLEMENT CONSIDERATIONS

- Ensure adequate omega-3 intake.
- Ensure adequate magnesium and zinc intake.
- Consider curcumin at 500–1,000 mg/day.
- Prioritize a diet rich in fruits and vegetables.

TOPICAL TREATMENTS

- Salicylic acid can help unclog pores and reduce acne.
- Benzoyl peroxide can help reduce acne-causing bacteria and inflammation.
- Topical tretinoin can help prevent clogged pores and treat acne over time.
- Niacinamide or green tea extract may provide additional anti-inflammatory support.

IF ACNE PERSISTS

- Review your testosterone dose with your TRT provider.
- Consider whether hormonal fluctuations or excessive testosterone exposure may be contributing.
- Consider discussing isotretinoin with a dermatologist if acne is severe, persistent, scarring, or doesn't respond to topical treatments and other standard therapies. It is one of the most effective treatments for severe acne and can produce prolonged remission after a course of treatment. Isotretinoin requires medical supervision because it can affect **lipids and liver function**, among other potential adverse effects. Baseline and follow-up blood testing are commonly used.

ESTROGEN CONTROL

If for any reason you need get side effects from too much estrogen (nipple sensitivity, water retention, erectile disfunction, etc), then you can use an aromatase inhibitor in the short term to alleviate the symptoms and while you change your drug selection:

Anastrozole	0.25-1mg daily
Or Nolvadex	10-20 mg daily

PART VI

NON-AAS COMPOUNDS

Not every goal requires simply increasing AAS exposure. This section covers some non-AAS compounds discussed in the framework.

HGH

Human growth hormone (HGH) is a natural protein hormone made by the pituitary gland at the base of the brain that controls physical growth, cell repair, and metabolism.

A 5-days-on, 2-days-off schedule is my preferred approach, which simply means using HGH Monday through Friday and then taking the weekends off.

HGH can cause water retention, and some users use a 5-on/2-off schedule in an attempt to manage that effect. The extent to which this preserves benefits while reducing side effects will vary between individuals.

Administration strategies:

1. **Hypertrophy/Growth:** If your main goal is growth, then administer HGH right before bed in the evening. Do not eat after your injection.
2. **Fat loss:** If your main goal is fat loss, then use HGH first thing in the morning, and do fasted cardio after administration. It could be paired with a cup of black coffee as well.

How to administer it: sub-q injection.

RECOMMENDED DOSE PER ADMINISTRATION	DOSE
AM (pre fasted cardio)	2 IUs
PM (pre bed)	2-4 IUs

MODAFINIL

It is a prescription wakefulness-promoting agent used to treat excessive daytime sleepiness. In the sports performance and physique enhancement world it can be used as an agent to improve mood and energy.

There are several use cases for modafinil. For example, it can be used during periods where you want to combat the diet induced fatigue, it can help fat loss in two ways. Number one, it seems to have an appetite suppressing effect, and number two it will increase energy expenditure since you will feel more energetic while taking it.

It will also improve your work capacity, which means you will be able to do more of a certain activity while experiencing less fatigue.

Personally, I also believe it can be used during periods of intense learning both for a physical skill, or mental skill. For example, if you practice a combat sport, you could use modafinil to increase the rate of learning new techniques and skills. On the other hand, if you want to get better at chess, you can also use modafinil to increase the rate of learning strategies.

Dosing strategies: 50–200 mg per day is a range sometimes used clinically, but dosing should follow the prescribing clinician's instructions. Taking it earlier in the day may reduce the chance of sleep disruption for some people.

I would also recommend taking it as early as possible to avoid sleep issues, and using it during specific periods of time, then taking a break. For example, you can use during a 16-week block to work on a specific skill, then take a break for 16 weeks or more before introducing it again.

PART VII

HEALTH PRESERVATION

HEALTH-PRESERVING IDEAS

First, if your health is gone, you cannot make more progress toward your goals. You have to be healthy enough to stay in the game.

The first thing you need to do is evaluate your current health status and make decisions when it comes to compound selection, dose, duration, training, and nutrition design in a way that aligns with your short-term goals, long-term goals, and risk level.

Additionally, here are some habits and supplements that will help you stay healthy:

RECOMMENDED LIFESTYLE HABITS

No amount of supplements will replace proper lifestyle habits. Here are some things that will have a major impact on your progress and health. Most people will ignore these; don't be one of them:

1. **Sleep:** 7–9 hours of high-quality sleep is recommended. If you find yourself tired every day after sleeping, consider a sleep study to discard the potential of sleep apnea.

2. **Fruits and vegetables:** make them a major part of your diet to support fiber, micronutrient intake, and overall diet quality.

3. **Cardio:** This is overlooked by most PED users because it requires additional effort, but cardio, along with the other lifestyle habits, is one of the best things you can do to preserve your health. The minimum I'd recommend is roughly 80–100 minutes of Zone 2 cardio per week, with 150 minutes per week as a useful target if recovery allows.

   As an example, when my body weight hit 220 lbs, my blood pressure spiked to 140/90. Instead of relying on other pharmaceuticals to lower it, I focused on increasing my cardiovascular capacity, and with cardio alone, I was able to bring it down to 115/75 without needing additional pharmaceutical interventions.

   Additionally, because I got very fit, my work capacity increased, and my mood, libido, and sleep got better—all of which allowed me to be a better athlete and healthier bodybuilder. Now, cardio is a staple in my week-to-week planning regardless of the phase I am in.

4. **Blood pressure:** Measuring your blood pressure a few times per week is a good idea, since using high doses of AAS tends to increase your blood pressure.

5. **Be conservative with your doses:** Most of your long-term progress comes from doing the right things consistently for years. That means you need to stay healthy enough to remain in the game. If you start using very high doses from the get go, your enhanced career might come to an end early. Be moderate with your doses, use enough to make progress, and really master everything else outside of AAS use.

RECOMMENDED HARM-REDUCTION SUPPLEMENTS

PED users will generally face some of the following: a poor lipid profile, high blood pressure, increased blood viscosity, and more.

The supplements below are the "starting point" for PED users as prophylactic measures. That being said, an individual protocol is appropriate to make sure you're looking after your health.

There are other supplements that could be implemented if necessary, but that needs to be evaluated on a case-by-case basis.

Strom Sports — US site, UK site (non-affiliate links)

AM

- Creatine monohydrate — 10 g per day
- MultiMax — Multivitamin and minerals
- SupportMax — Heart, liver, and kidney support
- LipidMax — Lipid support
- Omega-3 — 3–4 g of EPA + DHA per day

PM

- ZMAX — Sleep and recovery
- SupportMax — Inflammation and joint support
- NeuroMax — Sleep, recovery, and cognitive/brain health

If you can't access the Strom Sports brand, you can also get the separate ingredients. Here's a recommended list:

AM

- Omega-3 — 3–4 g of EPA + DHA per day
- Vitamin C — 1,000 mg
- NAC — 1,200 mg per day
- CoQ10 — 200–400 mg per day
- Green tea extract — 1–2 capsules per day
- Vitamin D3 — 3,000–5,000 IU per day
- Vitamin K2 (MK-7) — 100–200 mcg per day

PM

- ZMA
- Vitamin C — 1,000 mg

PART VIII

THE SMALLEST EFFECTIVE DOSE

ANABOLIC ANDROGENIC STEROID USE NEEDS TO BE PAIRED WITH GOOD HABITS

Taking the highest dose is not automatically the best way to make progress. Increasing drug exposure can increase results, but it can also increase adverse effects and health risks.

> **The goal should be to make the most progress possible on the smallest dose.**

Why?

Because the less you rely on drugs to create results, the more you're forced to maximize the things that support your health, performance, and long-term progress.

And the people that end up creating the best physiques are the ones that are able to do this for a very long time. Moderate to low androgen exposure for 10 years along with good habits will create a better physique than blowing your doses right out of the gate, destroying your health and then having to stop because you created some serious health issues for yourself.

These are the habits that can move the needle the most:

PROPER LIFESTYLE HABITS

Recovery and growth don't come from one thing. They're the result of several different pieces working together.

First, you need the right amount of training volume. More isn't always better. You want enough training to stimulate progress without creating unnecessary fatigue and beating yourself into the ground.

Second, you need a well-designed diet that matches both your short-term and long-term goals. Your nutrition should support your training, recovery, body composition, and overall health.

Third, you need to manage your stress. This doesn't have to be complicated. Ten minutes of meditation, going for a walk, spending time with your family, walking in nature, getting a massage, or doing something that brings you joy are great stress-management strategies.

And finally, in my opinion, one of the biggest things you can maximize is your sleep—both the amount and the quality. If you have trouble getting high-quality sleep, then this is the area I would focus on improving the most before you turn to more milligrams.

If your training, nutrition, stress management, and sleep are all dialed in, you can get significantly more out of a given dose.

Always think about how to maximize the results you can get from the minimum amount of drugs necessary.

PICKING YOUR DOSES

If you're just a recreational athlete without looking to compete in bodybuilding shows or having big aspirations to become a professional athlete or competitive bodybuilder, and your only goal is to develop a great-looking physique, then, in my opinion you can do that without going over 5-10 mg/kg in your cycles, I'd favor staying closer to 5 mg/kg.

As stated in the previous point, most of your progress will come from maximizing your lifestyle, training, and nutrition habits. Once you have these dialed in, then you'll be able to get a lot of juice out of smaller doses.

Also, progress with AAS, just like with fitness in general, will happen over a long period of time. So instead of focusing on taking more and more, focus on doing the right things for years. I know it doesn't sound sexy, but my goal here is to give you a framework that will allow you to achieve your fitness goals while preserving your health as much as possible.

USING AN AROMATASE INHIBITOR IS NOT IDEAL (UNLESS YOU NEED IT)

Aromatase inhibitors should be kept in your emergency kit in case you need them for an emergency.

But they shouldn't be part of your cycle from the beginning. Instead of using an AI, make better drug choices. If too much testosterone is giving you unwanted estrogenic effects, take less and/or replace it with another drug.

Maintaining optimal estrogen levels is essential for overall male health, as having levels that are either too high or too low causes severe physical and mental disruptions.

- **Muscle & Joints:** It repairs damaged muscle fibers, protects tendons and ligaments, and prevents joint pain and stiffness.
- **Mood & Brain:** It regulates serotonin and dopamine to prevent **depression, anxiety, and brain fog**.
- **Sexual Health:** It works alongside testosterone to maintain a healthy **libido and erectile function**.
- **Bone Density:** It is the primary hormone responsible for keeping male bones strong and preventing **osteoporosis**.
- **Heart & Metabolism:** It helps manage healthy cholesterol (HDL), blood flow, and insulin sensitivity.

Crushing your estrogen levels by using aromatase inhibitors can increase the deleterious health effects of using high doses of AAS.

One more caveat to add here is that every man will tolerate different levels of estrogen, so if you have elevated levels of estrogen in your blood panel but you're not experiencing negative side effects, then you wouldn't need to take measures to lower your estrogen.

PART IX

LONG-TERM REASSESSMENT

SHOULD YOU RUN SPORTS TRT INDEFINITELY?

You can do whatever you want.

But is it a smart choice to run a sports TRT/HRT dose indefinitely?

The answer depends on your goals, health, and individual circumstances. You need to look at the trade-offs and make a decision based on what you actually want from your body and your life.

I'll use myself as an example.

Personally, I don't want or need a sports TRT/HRT dose for the following reasons:

1. I already experimented with high doses of anabolics when I was younger, so I got that out of my system.
2. I'm predisposed to high blood pressure, and more anabolics can mean more water retention and potentially higher blood pressure. That isn't something I want to deal with.
3. I developed sleep apnea while using higher doses of anabolics. Even when it is controlled, additional body weight can make it harder to manage.
4. Increasing my AAS dose can increase body weight, which makes moving around harder, can contribute to sore joints, and it becomes a burden for my cardiovascular system.
5. Health and longevity are top priorities for me, so the additional exposure does not align with those goals.

This is my personal reasoning. Yours can—and probably should—be different. Make your own decisions based on your individual circumstances.

PART X

INJECTION GUIDE

SUBQ VS INTRAMUSCULAR INJECTIONS

There are pros and cons to each approach.

Benefits of SubQ:

- Less scar tissue builds up over IM administration.
- Improved adherence
- Lower barrier to entry due to needle size
- Better for TRT or daily doses where you're only injecting small volumes.

Disadvantages

- Large injection volumes will most likely be poorly tolerated and leave big lumps.

PERFORMING INTRAMUSCULAR INJECTIONS

- How to self-administer: if you're using multiple compounds, you can draw them into the same syringe, just make sure to disinfect each vial before drawing.
- Injection sites:

  Dorsogluteal · Ventrogluteal · Deltoid · Quad
- Injection site rotation: rotating 4-6 injection sites is ideal to prevent scar tissue formation.

  Example: right delt, left delt, left ventrogluteal, right ventrogluteal.
- Injection frequency: Splitting your into 2-3 injections per week is ideal.

NEVER INJECT AN AREA THAT IS:

- Red
- Hot
- Swollen
- Bruised
- Irritated
- Painful
- Hardened
- Scarred
- Broken
- Showing signs of infection

If an injection site looks or feels significantly different from normal, choose another approved site.

Contact your healthcare professional if you are concerned.

PART XI

THE BIG PICTURE

THE PED DECISION-MAKING FRAMEWORK

If you remember nothing else from this guide, remember these questions:

1. **What are you trying to accomplish?**
2. **What level of androgen exposure is actually necessary for that goal?**
3. **What is your timeline?**
4. **What does your health look like before you start?**
5. **What happens to your health markers while you're using it?**
6. **Are training, nutrition, sleep, cardio, stress management, and recovery actually optimized?**
7. **Are you getting enough benefit from the current dose to justify increasing it?**
8. **What happens when you reassess?**

In my opinion, the goal should be to make the most progress possible on the smallest dose.

If your training, nutrition, stress management, and sleep are all dialed in, you can get significantly more out of a given dose.

If you have a terrible diet, are sleeping 4–5 hours per night, and drinking alcohol a few times per week, then taking more will just wreck you more.

> **That's the goal: maximize the results you can get from the minimum amount of drugs necessary.**

DO YOU WANT ADDITIONAL HELP?

If you want me to personally help you build a plan around your goals, manage your health markers, and design your training and nutrition, you can **apply for coaching here**.